

Content Strategy

DHS Multispecialty Hospital — www.dhshospitals.com

A 6-month plan for SEO, AI Overviews, and local Ahmedabad reach

Prepared by **NovaBuildBot's SEO Bot** · an AI agent from **Nova AI Technologies LLP**

Prepared for DHS Multispecialty Hospital	Prepared by NovaBuildBot — SEO Bot agent a product of Nova AI Technologies LLP	Report date May 07, 2026 For review & approval
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1. Executive Summary

DHS Multispecialty Hospital has a strong technical SEO foundation and a high-value differentiator (VELYS robotic knee replacement). The next growth lever is **consistent, doctor-validated editorial content** that captures patient questions before they decide where to seek care.

NovaBuildBot is built for exactly this. DHS does not need to hire a content writer, an SEO consultant, or a digital marketing lead. Our SEO Bot agent drafts, edits, optimises, publishes, and promotes every post end-to-end. The only thing we ask of DHS is a **10–15 minute medical-accuracy review** from one specialist on the relevant topic, before each post goes live. The buck stops at medical accuracy — everything else is on us.

This document proposes a **pillar-and-cluster** content model, three non-negotiable editorial rules, a 6-month weekly editorial calendar, and a recommended first follow-up post. It is intended for review with the DHS leadership team before publication begins.

Why now

Two industry shifts make this the right moment:

1. Google's AI Overviews and ChatGPT/Perplexity now answer many patient questions before any link is clicked. Ranking in the AI summary depends on having clean, question-shaped, doctor-authored content indexed against your hospital domain.
2. Local search competition in Ahmedabad orthopedics is rising. The hospitals that publish first capture the long-tail informational queries (e.g. "knee pain old age", "is robotic surgery safer") that feed the booking funnel.

2. Where We Are Today

Already shipped (May 2026)

Service pillar pages — strong, well-linked, schema-rich:

- **/velys-robotic-knee-replacement-ahmedabad/** — exact-match landing for the highest-intent VELYS query, with comparison table, recovery cards, both surgeon profiles, and 9 FAQs
- **/departments/robotic-surgery/** — robotic joint replacement hub with 10 question-shaped FAQs
- **/departments/joint-replacement/**, **/departments/orthopedics/**, all 21 clinical department pages
- **/doctors/doctor-hardik/**, **/doctors/doctor-swagat/** — Physician schema, expertise listed

Blog posts — one launch announcement live:

- **/blog/robotic-knee-replacement-ahmedabad/** — VELYS launch (May 2026)

The gap

The site's content funnel is currently **bottom-heavy**. We have transactional pages (book appointment, doctor profiles, the launch announcement) but very little **top-of-funnel** content — articles that capture patients while they are still researching symptoms and haven't decided where to seek care.

Top-of-funnel content is what AI Overviews quote, what featured snippets pull from, and what eventually drives a measurable share of appointment bookings 60–90 days after publication.

3. Strategy — Pillar & Cluster

Each major service is a **pillar page**. Around it, we publish **cluster posts** that answer specific patient questions and link back to the pillar. This signals topical authority to Google and gives AI engines a connected knowledge graph to draw from.

How a cluster looks

Pillar — **VELYS Robotic Knee Replacement**

- Cluster post — Knee pain in older adults: when to consider replacement
- Cluster post — Robotic vs traditional knee replacement
- Cluster post — What to expect the day of your knee replacement
- Cluster post — Knee replacement recovery: a week-by-week timeline
- Cluster post — Cost & insurance for robotic knee surgery in India

Three editorial rules — non-negotiable

1. Question-shaped title

Every post title is phrased as the patient's actual query. *"How long does knee replacement recovery take?"* beats *"Knee Replacement Recovery"* on AI Overviews and featured snippets, and reads more naturally when shared on WhatsApp. Use the same phrasing as the H1 — they should match.

2. Doctor by-line + medical reviewer

Healthcare content is YMYL (Your Money or Your Life) — Google's highest E-E-A-T bar. Every post must be authored by a credentialed DHS specialist and reviewed by a second physician. The author block is rendered with credentials (FRCS, MS, DNB, etc.) and links to the doctor's profile page so search engines can verify identity.

3. One pillar, one location

Each post links to at least one service pillar (VELYS, Joint Replacement, Cardiology, etc.) and mentions "Ahmedabad" plus a specific neighbourhood (Vastrapur, Gurukul, Drive-In, Satellite, Bodakdev, Memnagar, Thaltej, Sabarmati, Naranpura). Locality lives in body copy, breadcrumbs, and structured data — never in the title alone.

4. Search Intent Ladder

Patients move through three search stages before booking. We need content at each stage:

Stage	What the patient asks	Page type	Where DHS stands
Top of funnel (information)	"Why does my knee hurt?" "What is a slipped disc?" "Heart attack warning signs"	Symptom & condition guides written by specialists	Almost no coverage — biggest opportunity
Middle of funnel (comparison)	"Robotic vs traditional knee replacement" "Cost of bypass surgery"	Comparison guides, decision aids, recovery timelines	Partial — VELYS landing covers one comparison
Bottom of funnel (transactional)	"Best knee surgeon Ahmedabad" "Robotic surgery near me"	Pillar landing pages, doctor profiles, booking flow	Strong — VELYS landing, doctor pages, schema

Implication: The next 4–5 posts should focus on top-of-funnel and middle-of-funnel content. This balances the funnel and feeds qualified traffic into the existing transactional pages.

5. 6-Month Editorial Calendar — Weekly

Cadence: one post per week, 24 posts in 6 months. NovaBuildBot drafts each post end-to-end; the assigned DHS specialist reviews for medical accuracy in roughly 10–15 minutes before publish.

Two-tier post mix

Cornerstone (every other week, 12 posts) — long-form 1,200–1,800 word patient guides that anchor a service pillar.

Mid-weight (every other week, 12 posts) — focused 600–900 word posts: comparison guides, plain-English explainers, condition primers, patient FAQs. Same E-E-A-T standard, faster to read, faster to review.

Week	Post (working title)	Tier	Funnel	Pillar	Reviewer
W1 ■	VELYS Robotic Knee Replacement now in Ahmedabad (launch)	Cornerstone	BoF	VELYS	Dr. Hardik
W2	Knee Pain in Older Adults — When to Consider Replacement	Cornerstone	ToF	Joint Replacement	Dr. Hardik
W3	Robotic vs Traditional Knee Replacement — Quick Comparison	Mid-weight	MoF	VELYS	Dr. Hardik
W4	What to Expect on the Day of Your Knee Replacement	Cornerstone	MoF	Joint Replacement	Dr. Swagat
W5	5 Questions to Ask Your Knee Surgeon	Mid-weight	MoF	Joint Replacement	Dr. Hardik
W6	Knee Replacement Recovery — A Week-by-Week Timeline	Cornerstone	MoF	VELYS	Dr. Swagat
W7	ACL Tears in Athletes — Symptoms, Surgery & Return to Sport	Cornerstone	ToF	Sports Medicine	Dr. Swagat
W8	Why Indians Are Switching to Robotic Surgery — Patient FAQ	Mid-weight	MoF	VELYS	Dr. Hardik
W9	Trauma & Accident Care — When to Come to DHS Emergency	Cornerstone	ToF	Trauma + ICU	Dr. Hardik
W10	Cost & Insurance for Robotic Knee Surgery in India	Mid-weight	MoF	VELYS	Dr. Hardik

Week	Post (working title)	Tier	Funnel	Pillar	Reviewer
W11	Heart Attack Warning Signs You Shouldn't Ignore	Cornerstone	ToF	Cardiology	Dr. Sunil
W12	Hip Replacement vs Knee Replacement — Which Comes First?	Mid-weight	MoF	Joint Replacement	Dr. Hardik
W13	Spine Surgery — When Is Conservative Care No Longer Enough?	Cornerstone	ToF/MoF	Spine Surgery	Dr. Hardik
W14	Diabetes & Joint Pain — The Hidden Connection	Mid-weight	ToF	Internal Medicine	Dr. Archit
W15	Cancer Surgery in Ahmedabad — Choosing a Surgical Oncologist	Cornerstone	MoF	Cancer Care	Dr. Dileep
W16	Reading Your Knee MRI Report — A Patient's Plain-English Guide	Mid-weight	ToF	Radiology	Dr. Yesha
W17	Hernia Repair — Laparoscopic vs Open Surgery Compared	Cornerstone	MoF	General Surgery	Dr. Chirag
W18	Stroke Care in Ahmedabad — The Golden Window	Mid-weight	ToF	Neurosurgery	Dr. Kushal
W19	Kidney Stones — Symptoms, Treatment & Prevention	Cornerstone	ToF	Urology	Dr. Darshil
W20	Pre-op Anaesthesia — What Patients Worry About (And Shouldn't)	Mid-weight	MoF	ICU	Dr. Mansi
W21	Gallbladder Stones — Surgery vs Watch-and-Wait	Cornerstone	MoF	Gastroenterology	Dr. Ruchir
W22	Why Sleep Matters After Major Surgery	Mid-weight	ToF	Internal Medicine	Dr. Archit
W23	Choosing Between OPD and Hospital Admission — A Quick Guide	Mid-weight	ToF	Internal Medicine	Dr. Archit
W24	Patient Stories — A Year of Robotic Knee Replacement at DHS	Cornerstone	BoF	VELYS	Dr. Hardik

Reviewer column = the DHS specialist NovaBuildBot will share the draft with for a ~10-minute medical accuracy check. Adjust the assignment freely; we will follow the doctor pool DHS provides.

6. Recommended Next Post (Post #2)

Working title: Knee Pain in Older Adults — When Should You Consider Knee Replacement?

URL	/blog/knee-pain-when-to-consider-knee-replacement-ahmedabad/
Tier	Cornerstone
Funnel	Top of funnel (information)
Target queries	"knee pain old age", "knee pain treatment Ahmedabad", "when do I need knee replacement", "is my knee pain serious"
Pillar link	/departments/joint-replacement/ → /velys-robotic-knee-replacement-ahmedabad/
Drafted by	NovaBuildBot SEO Bot — full draft in Dr. Hardik's voice, with schema, locality, citations
Medical reviewer	Dr. Hardik M Shah (MS Orthopedic, FRCS Germany) — ~15 min accuracy review
Estimated length	1,200–1,500 words
Hero image	Original photo of an outpatient knee assessment (recommended)

Why this post specifically

- Highest search volume.** "Knee pain", "knee pain in old age", and "knee pain treatment" are massive queries in India. Approaching the topic from a symptom-first angle captures patients *before* they have decided on surgery — much earlier in the decision journey than "knee replacement" alone.
- Funnels naturally to your existing pillars.** The post will conclude by pointing readers who have decided to consider surgery toward the Joint Replacement and VELYS landing pages.
- E-E-A-T friendly.** Symptoms and red-flag content is exactly what Google rewards when written by a credentialed surgeon — the "experience" and "expertise" signals are obvious.
- AI Overview bait.** Patients ask AI assistants "is my knee pain serious?" daily. Question-shaped subheadings ("When is knee pain just ageing?", "Red flags that mean you should see a specialist") are the exact paragraph shapes AI engines quote.
- Local SEO win.** "Knee pain Ahmedabad" has clear commercial intent and no dominant ranking page yet — a clean ranking opportunity in 60–90 days.

Proposed outline

- Why knees ache as we age (the basic biomechanics)
- When is knee pain just ageing — and when is it something more?

3. Red flags that mean you should see a specialist
4. Conservative options doctors try first (medication, physio, injections)
5. When is knee replacement the right answer?
6. What changes with robotic-assisted knee replacement
7. How DHS Multispecialty Hospital approaches knee care in Ahmedabad
8. FAQs (5–6 question-shaped, AI-quotable)
9. CTA — book a consultation

7. Measurement & KPIs

Content marketing pays off in 60–120 days, not 60–120 hours. We need the right metrics so leadership can judge progress without false signals.

What we will measure (monthly)

- **Organic impressions** for target keyword clusters (Google Search Console)
- **Average position** for the 5–10 priority queries per pillar
- **Organic clicks** to each blog post and to its linked pillar page
- **Appointment-form conversions** attributed to blog landings (GA4 event)
- **AI Overview / featured-snippet wins** for tracked queries (manual quarterly check)
- **Backlinks earned** per post (Ahrefs / free SEO tools)

What we will not over-index on

- **Page views in week 1.** Most patient-research content takes 6–12 weeks to rank, then climbs steadily.
- **Bounce rate.** Informational content has higher bounce by design — readers got their answer.
- **Social shares.** Hospital content is rarely shared; that's not the channel.

Realistic targets (12-month horizon)

Total organic sessions	+150% over baseline
Branded queries (e.g. "DHS Hospital")	+50%
Non-branded informational queries	+300% (largest growth)
Blog → appointment conversion rate	1.5%–3% (industry benchmark for hospitals)
AI Overviews citing DHS for ortho queries	≥3 captured per quarter
Top-3 ranking on at least 5 priority queries	by month 9

8. How This Works — DHS Reviews, NovaBuildBot Ships

DHS does not need to hire writers, SEO specialists, content strategists, or a digital marketing lead. NovaBuildBot's SEO Bot agent does that work end-to-end. The only thing we ask of DHS is **medical accuracy review** — typically 10–15 minutes per post, from any one specialist on the relevant topic.

Who does what

Topic research & SEO	NovaBuildBot — keyword analysis, search-intent mapping, competitor gap analysis
Drafting	NovaBuildBot — full draft in DHS doctors' voice, with locality, schema, citations
Editorial polish	NovaBuildBot — SEO, readability, AI-Overview formatting, house style
Medical accuracy review	One DHS specialist (~10–15 min) — verifies clinical facts & safety. The buck stops here.
Final sign-off	Medical Director (Dr. Hardik Shah) — short approval before publish
Publishing	NovaBuildBot — to /blog/, schema, sitemap, search console, internal links
Distribution	NovaBuildBot — Instagram + WhatsApp + cross-link to drhardikshahortho.com
Performance reporting	NovaBuildBot — monthly KPI report to DHS leadership

Per-post process — total DHS time: ~15 min

1. NovaBuildBot picks the next post from the agreed calendar, drafts it end-to-end
2. Draft is shared with the assigned DHS specialist (the topic owner)
3. Specialist reviews for medical accuracy — flags anything to correct (~10–15 min)
4. Medical Director signs off (single tap/email)
5. NovaBuildBot publishes, indexes, cross-links, schedules Instagram + WhatsApp

What this means in practice

At the proposed cadence (1 post / week), DHS doctor review time totals roughly **1 hour per month**, distributed across whichever specialists are most appropriate per topic. There is no marketing team to hire, no SEO consultant to onboard, no editorial workflow tools to set up. NovaBuildBot brings all of that to the table as part of the engagement.

Distribution beyond search

Publishing the post is half the job. Each post should also be:

- **Promoted on Instagram** — short-form summary post, swipe carousel of key takeaways
- **Sent on WhatsApp** — to the hospital's existing patient list with a one-line teaser
- **Cross-linked from Dr. Hardik's external site** (drhardikshahortho.com) — bidirectional backlink boosts both domains
- **Submitted to Google Search Console** — "Request indexing" on day of publish for fastest pickup
- **Featured in the next month's email newsletter** if/when one launches

Decision needed before we ship Post #2. Please confirm:

- Approval of the pillar-and-cluster strategy and the weekly 6-month calendar (24 posts)
- A small **review pool** of 3–5 DHS specialists who can each take a 10–15 minute medical-accuracy review when their topic comes up
- Final sign-off path — single Medical Director sign-off (Dr. Hardik Shah) is the simplest workflow unless a broader committee is preferred
- Confirmation that NovaBuildBot may publish to /blog/, request indexing, and run Instagram + WhatsApp distribution on DHS's behalf

About this document. This content strategy was prepared by **NovaBuildBot's SEO Bot agent** on May 07, 2026 for DHS Multispecialty Hospital. NovaBuildBot is a product of **Nova AI Technologies LLP** — AI agents that ship SEO and content work end-to-end, with human strategist oversight. For questions, revisions, or to commission additional work, reach out via the same channel that delivered this document.